

THE CYCLE SPACE

Know Your Cycle

**A practical, clinically grounded
guide to hormones, symptoms and
fertility signs**

For women who want clarity, not confusion.



EBOOK | BODY LITERACY

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THE CYCLE SPACE | BODY LITERACY EBOOK

01

medical safety note

Before we begin

This guide is educational. It helps you observe patterns, ask better questions and prepare for personalised care. It does not diagnose, treat, prevent or cure disease, and it does not replace urgent medical care or your clinician's advice.



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Your cycle is not background noise.

A period is not just a monthly event. It is the visible part of a larger hormonal rhythm involving the brain, ovaries, uterus, thyroid, stress system, sleep, nutrition and metabolism.

Expert lens

ACOG describes the menstrual cycle as a useful vital sign, because patterns in timing, bleeding and symptoms can offer meaningful information about health. The Office on Women's Health also notes that regular menstrual periods between puberty and menopause are usually a sign the body is working normally, while irregular, heavy or painful periods deserve attention. [1,2]



Rhythm

Cycle length, regularity, missed periods and sudden changes.



Flow

Bleeding length, heaviness, clots, spotting and flooding.



Signals

Pain, mood, sleep, skin, digestion, energy and libido.

A better question than 'Is this normal?'

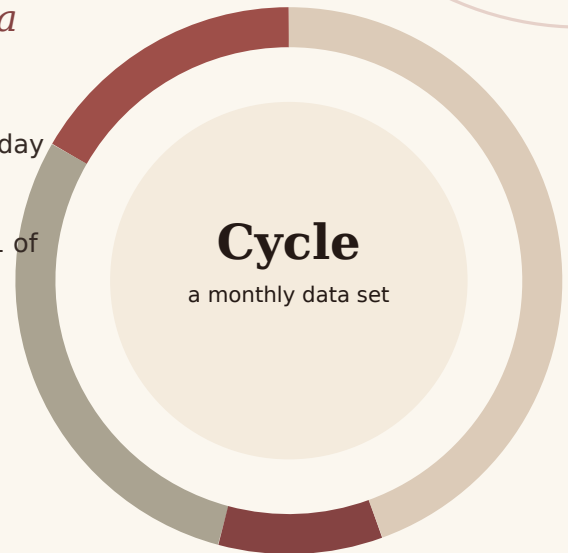
Ask: Is this normal for me, and has it changed? The goal is not to obsess over every data point. The goal is to notice your body's pattern early enough to respond with curiosity rather than panic.

What is a typical cycle?

For many adults, menstrual bleeding happens roughly every 21 to 35 days and lasts about 2 to 7 days. Some sources use a 24 to 38 day range for adults. Adolescents often have more variability, especially in the first years after menarche. [1,3,4]

Use typical ranges as orientation, not as a verdict.

- Day 1 is the first day of proper menstrual bleeding, not a day of light pre-period spotting.
- Cycle length is counted from Day 1 of one period to Day 1 of the next.
- Ovulation usually happens before the next period, not always on Day 14.
- Change matters. A sudden new pattern is often more relevant than a single imperfect cycle.



The Cycle Space principle

Your cycle is a monthly data set. You do not need to be perfect at tracking. You need enough information to recognise your own rhythm.

The four phases, made simple.

Every cycle is a conversation between hormones. Estrogen tends to rise before ovulation. Progesterone rises after ovulation. These shifts can influence energy, mood, appetite, cervical mucus, temperature and sleep.

01

Menstrual

The bleed. Energy may be lower. Pain is common, but severe pain that disrupts life deserves care.

02

Follicular

Estrogen often rises. Many women feel more energy, clearer thinking or more motivation.

03

Ovulatory

The fertile window. Cervical mucus often becomes wetter, clearer, stretchier and more slippery.

04

Luteal

Progesterone rises after ovulation. Temperature usually rises. PMS can appear if the body is under strain.

Important nuance

Cycle phases are not personality types. They are physiology. Use them to understand patterns, not to limit yourself.

What each hormone is broadly doing.

This page gives a simple orientation. It is not a lab interpretation.



FSH

Helps recruit ovarian follicles early in the cycle.



Estrogen

Builds the uterine lining and supports fertile cervical mucus before ovulation.



LH

Surges before ovulation and helps trigger the release of an egg.



Progesterone

Rises after ovulation, supports the uterine lining and raises basal body temperature.



Prostaglandins

Help the uterus contract. High levels can contribute to cramping and period pain.



Cortisol and insulin

Stress and blood sugar regulation can influence ovulation, appetite, sleep and PMS patterns.

Fertility is visible if you know what to observe.

Fertility awareness does not mean guessing by an app prediction. It means observing biological signs such as cervical mucus, basal body temperature and sometimes ovulation tests. ACOG notes that fertility awareness methods involve tracking menstrual cycle signs to estimate fertile days. [5]

The core idea

The fertile window is not the whole month. ASRM notes that fertility-awareness methods such as ovulation tests and cervical mucus monitoring can increase the probability of conceiving in a given cycle when used to time intercourse. [6]

01

Cervical mucus

Often becomes wet, slippery and stretchy near ovulation. This supports sperm movement and survival.

02

Temperature

Basal body temperature usually rises after ovulation because progesterone has a thermogenic effect.

03

LH tests

Ovulation tests can detect an LH surge before ovulation. They are useful, but not the whole story.

Contraception note

Do not use fertility awareness to avoid pregnancy unless you have been properly taught a method and understand its rules. Apps alone are not a method.

A practical cervical mucus map.

Cervical mucus changes through the cycle. Wet, slippery mucus is commonly associated with higher fertility, while dry or sticky patterns are usually less fertile. ACOG and Cleveland Clinic both describe cervical mucus observation as a useful fertility sign. [5,7]

Pattern	Possible meaning	How it may feel
Dry or little mucus	Lower fertility	Dry, no sensation, or very little visible mucus
Sticky or tacky	Lower to rising fertility	Thick, pasty, crumbly or sticky
Creamy	Fertility may be rising	Lotion-like, smooth, opaque
Wet, slippery, stretchy	High fertility	Lubricative, clear or egg-white-like
Sudden dry-up after peak	Ovulation likely passed	Mucus reduces as progesterone rises

Expert caution

Mucus can be altered by infections, medication, hydration, breastfeeding, perimenopause and hormonal contraception. If discharge has a strong odour, itching, burning, pain or unusual colour, seek medical advice.

Track symptoms as information, not identity.

Symptoms are not moral failures. They are patterns. The goal is to connect symptom timing with cycle phase, stress, sleep, food, movement, digestion and life context.



Pain

Cramping, pelvic pain, pain with sex, bowel pain, ovulation pain.



Bleeding

Heavy bleeding, clots, flooding, spotting, bleeding after sex.



Mood

Anxiety, irritability, sadness, rage, low motivation, PMDD-type symptoms.



Energy

Fatigue, insomnia, low resilience, afternoon crashes.



Skin

Acne patterns, especially jawline or luteal-phase flares.



Digestion

Bloating, constipation, diarrhoea, nausea, cravings.

Pattern question

Does this happen before bleeding, during bleeding, around ovulation, after ovulation or randomly?

Pain is common. Being dismissed should not be.

Severe or worsening symptoms deserve assessment. ACOG advises evaluation for secondary causes such as endometriosis when dysmenorrhea does not improve after initial treatment. NICE highlights that pain with heavy menstrual bleeding may be caused by endometriosis and should be considered in assessment. [8,9]

- Period pain stops normal life, school, work, sleep or relationships.
- Pain is getting worse over time or starts later after years of easier periods.
- Heavy bleeding floods through protection, causes dizziness, fatigue or suspected anaemia.
- Bleeding after sex, bleeding between periods or bleeding after menopause.
- Cycles suddenly change, become very irregular, very long, very short or absent.
- Pelvic pain with fever, fainting, severe one-sided pain or pregnancy possibility needs urgent care.



The sentence to use with a clinician

“This symptom is affecting my daily life. I would like to understand the possible causes and what evaluation is appropriate.”

Common patterns worth investigating.

This is not a diagnostic page. It is a language page, designed to help you bring clearer information to a clinician.



PCOS

Often involves ovulatory dysfunction, clinical or biochemical hyperandrogenism, and polycystic ovarian morphology, with diagnosis



Thyroid dysfunction

Can affect cycle regularity, bleeding patterns, energy, mood, weight, temperature and fertility.



Fibroids or polyps

Can contribute to heavy bleeding, pressure symptoms, prolonged bleeding or bleeding between periods.



Endometriosis

Can involve period pain, chronic pelvic pain, pain with sex, bowel or bladder symptoms, and fatigue. Symptoms do not always match



PMDD

A severe cyclical mood disorder where symptoms occur in the luteal phase and improve after bleeding begins.



Bleeding disorders

Can be relevant with very heavy periods, especially since adolescence or with easy bruising or prolonged bleeding.

Hormonal health is not a hack.

The cycle is influenced by the whole body. The aim is not to control every variable. It is to create conditions that make ovulation, recovery, digestion and nervous system regulation more possible.



Sleep

Regular sleep supports circadian rhythm, appetite, stress regulation and recovery.



Blood sugar

Protein, fibre and enough food can reduce energy crashes and intense cravings.



Stress physiology

The nervous system is not separate from hormones. Chronic strain can affect ovulation and symptoms.



Movement

Strength, mobility, walking and cycle-aware rest can support metabolism and pain modulation.



Gut health

Digestion, inflammation, bowel regularity and the gut-vaginal axis can influence wellbeing.



Nourishment

The Cycle Space philosophy is additive: what can we add to support the body?

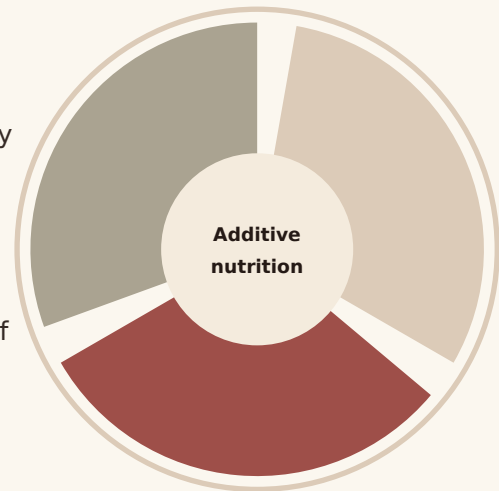
No fear-based wellness

This is not about earning a better cycle. It is about removing unnecessary confusion and adding support where your body is asking for it.

A cycle-supportive plate, without diet culture.

Nutrition here is not restriction. It is information and support. For fertility, ASRM notes there is insufficient evidence that one specific diet or macronutrient intake improves natural fertility, while folic acid supplementation before pregnancy reduces neural tube defect risk. [6]

- Protein to support satiety, tissue repair and blood sugar stability.
- Fibre and colour to support digestion, microbiome diversity and estrogen metabolism.
- Carbohydrates as fuel, especially around training, stress and the luteal phase.
- Healthy fats for hormone building blocks and absorption of fat-soluble nutrients.
- Enough food, because under-fuelling can be a stressor.



Your first month of tracking.

Keep it light. Track five things daily or near-daily. You are looking for patterns, not perfection.

01

Bleeding

None, spotting, light, medium, heavy, clots.

02

Mucus

Dry, sticky, creamy, wet, slippery, stretchy.

03

Temperature

Optional. Most useful when taken before getting out of bed.

04

Symptoms

Pain, mood, digestion, energy, skin, sleep.

05

Context

Stress, travel, illness, alcohol, intense training, big life events.

Tiny tracking rule

If tracking makes you anxious, simplify. One line per day is enough.

Cycle snapshot worksheet

Print this page or recreate it in your notes app.

Cycle start date	Cycle length	Bleeding days
Heaviest day	Pain score 0-10	Main symptom
Best energy days	Lowest energy days	Mucus peak day
Possible ovulation signs	Sleep pattern	Stress level

What changed this month?

Bring the pattern, not just the pain.

A good consultation becomes easier when your story is organised. You do not need to arrive with perfect answers. You need a clear starting point.



Timeline

When did this start? Was there a trigger, medication change, contraception change, pregnancy, loss, infection, stressor or move?



Cycle pattern

Average length, bleeding days, heaviest day, spotting and missed periods.



Top three symptoms

What affects your life most, and when in the cycle does it happen?



Medical context

Diagnoses, medication, supplements, contraception history, surgeries, tests, family history.



Goals

Less pain, fertility clarity, coming off the pill, PCOS support, perimenopause support, or simply understanding your body.

Clinical clarity, human warmth.

The brand's core promise is simple: women deserve to understand their bodies deeply, not merely manage symptoms well enough to keep going.

01

Listen

Your symptoms and lived experience are data.

02

Map

We place symptoms in time: cycle phase, life context, stress, sleep, digestion and nutrition.

03

Explain

Complexity is translated into language you can actually use.

04

Support

Recommendations are practical, sustainable and personalised.

What this is not

- A magic supplement protocol.
- A diet plan or weight loss programme.
- A replacement for urgent medical assessment.
- A place where pain is minimised.

A few words made simpler.

Use these terms as orientation, not as labels to self-diagnose.



Ovulation

Release of an egg from the ovary. It is the event that separates the follicular and luteal



Anovulation

A cycle where ovulation does not occur. Bleeding can still happen.



Amenorrhea

Absence of periods.



Luteal phase

The phase after ovulation, when progesterone should be higher.



Dysmenorrhea

Painful periods. Primary means without obvious pelvic pathology. Secondary means



Menorrhagia or HMB

Heavy menstrual bleeding that affects quality of life or causes clinical concern.

closing note

Your body is not the problem. It is a system to understand.

Start with observation. Add context. Ask better questions. Get support when your symptoms affect your life. This is the beginning of body literacy.

References and expert sources

1. American College of Obstetricians and Gynecologists. Menstruation in Girls and Adolescents: Using the Menstrual Cycle as a Vital Sign.
2. Office on Women's Health. Menstrual Cycle.
3. Mayo Clinic. Menstrual cycle: What's normal, what's not.
4. Office on Women's Health. Your menstrual cycle.
5. American College of Obstetricians and Gynecologists. Fertility Awareness-Based Methods of Family Planning.
6. American Society for Reproductive Medicine. Optimizing natural fertility: a committee opinion, 2022.
7. Cleveland Clinic. Cervical Mucus: Chart, Stages, Tracking and Fertility.
8. American College of Obstetricians and Gynecologists. Dysmenorrhea and Endometriosis in the Adolescent, Committee Opinion No. 760.
9. NICE. Heavy menstrual bleeding: assessment and management, NG88.
10. 2023 International Evidence-based Guideline for the Assessment and Management of Polycystic Ovary Syndrome.

Medical safety reminder

This ebook is educational and should not be used as a diagnostic tool, contraceptive instruction or substitute for medical care. Seek urgent help for severe acute pain, fainting, heavy bleeding with dizziness, suspected pregnancy complications, fever, or any symptom that feels unsafe.